

# How to Open an In-Home Care Agency in Ohio

*A step-by-step guide to getting your Non-Medical Home Health Services license from the Ohio Department of Health — screening and training caregivers, finding clients, and getting paid*

PolicyReady.org — Practical Playbooks for Care Entrepreneurs

## The Ohio difference — read this first

**Ohio licenses non-medical home care.** Since October 2022, anyone who provides personal care and homemaking in a client's home — or who is routinely alone with a client or has access to a client's property or money — needs a license from the Ohio Department of Health before serving clients. The license you want is the Non-Medical Home Health Services license. Unlike some states, Ohio does not split it into tiers or classes: it is one non-medical license covering personal care, homemaking, respite, and helping a client take their own medication. Skilled nursing and therapy are a separate, higher license you do not need for this business. This guide walks you through that license, the caregiver background checks Ohio is strict about, how you find clients, and how you get paid — privately and through Ohio's Medicaid waivers. Ohio also has no statewide sales tax on these services.

## How to Use This Guide

This guide takes you from idea to a licensed, operating Ohio in-home care agency. Read it once start to finish, then use it as a checklist. Everything here is general information for prospective Ohio agency owners — not legal, tax, or licensing advice. Requirements, fees, and phone numbers change, so verify each step with the agency named before you rely on it. Where this guide gives a fee or a figure, treat it as a starting point to confirm, not a final answer.

## The one-paragraph version

Form an Ohio business and get an EIN. Put your insurance, your surety bond, and a policy and procedure manual in place, and apply to the Ohio Department of Health for a Non-Medical Home Health Services license — the application takes owner fingerprints, a criminal-records-check policy, a description of your services, the bond, and a \$250 fee, and ODH suggests allowing about ninety days. Build your caregiver pipeline: every caregiver clears an Ohio Bureau of Criminal Investigation background check (requested within five business days of a conditional hire), is free of a disqualifying offense, completes your orientation and training, and is found competent before serving a client. Once your license is issued, start serving clients — privately at first, then through Ohio's Medicaid home-care waivers (PASSPORT, the Ohio Home Care Waiver, and MyCare Ohio) once you enroll as a provider, which is a separate step from your license and may be subject to a state moratorium.

## The Path at a Glance

Ten steps take you from idea to an operating Ohio in-home care agency:

1. **Form the business.** Register an LLC or corporation with the Ohio Secretary of State; get an EIN; open a business bank account.
2. **Name an administrator.** Designate the person responsible for running the agency and its compliance with ODH.

3. **Put protection in place.** Professional and general liability insurance, workers' compensation, and the surety bond the license requires.
4. **Write your policies.** Adopt the policy and procedure manual, client forms, and caregiver handbook the license and good practice require.
5. **Apply to ODH.** Submit the Non-Medical Home Health Services license application, fee, bond, owner fingerprints, and criminal-records-check policy.
6. **Build caregiver screening.** BCI (and FBI) background checks, database reviews, and the applicant-and-employee roster.
7. **Train and evaluate caregivers.** Orientation, initial training, and a competency evaluation before independent service.
8. **Get your license.** Allow about ninety days; do not serve clients until ODH issues the license.
9. **Start serving clients.** Begin with private pay; deliver, document, and supervise every visit per the service plan.
10. **Add Medicaid and grow.** Enroll as a PASSPORT / Ohio Home Care / MyCare provider (verify current availability), and build referral relationships.

## Part 1 — What This Business Is

An in-home care agency sends caregivers into clients' homes to help with everyday life: bathing, dressing, grooming, toileting, transferring, and eating (personal care); cleaning, laundry, meals, and errands (homemaking and supportive services); companionship and respite; and helping a client take their own medication. In Ohio this is a licensed activity — you are not just starting a business, you are opening a state-licensed care agency, and that shapes how you hire, train, and get paid.

Ohio is a large, aging market with strong demand for keeping older adults and people with disabilities at home. There is a premium private-pay market and a set of Medicaid waivers that fund in-home care. Ohio also has no statewide sales tax, which simplifies pricing. The trade-off for entering a licensed market is real: you clear a genuine licensing bar before you open, and you operate under the possibility of ODH review — but that bar keeps out unserious competitors and gives licensed agencies credibility with the hospitals, case managers, and families who choose them.

Practically, being licensed changes how you run day to day. You cannot quietly start with one client and sort out compliance later — the license comes first, and the systems that support it (screened caregivers, written service plans, supervision, and records) have to exist before you open. That is more work up front, but it means the systems that protect your clients and your business are built in from the start rather than bolted on after a problem. Think of the license as the operating discipline that lets you scale: an agency that is genuinely review-ready can add caregivers and clients without the wheels coming off.

### Is this business right for you?

Opening an in-home care agency in Ohio rewards people who are organized, patient with paperwork, and genuinely good with people — the job is equal parts care and compliance. You do not need a clinical background to own a non-medical agency; you do need an administrator who will own the compliance, enough capital to cover several months before revenue arrives, the temperament to recruit and keep caregivers in a competitive labor market, and the discipline to keep records a reviewer could open at any time. If that sounds like you, Ohio's licensed model is an advantage, not an obstacle: it keeps casual competitors out and rewards operators who take the work seriously.

### The service lines, in plain terms

- **Personal care:** hands-on help with the activities of daily living — the core of the non-medical license.
- **Homemaking and supportive services:** housekeeping, laundry, meals, shopping, and errands that keep a home safe and livable.
- **Respite and companionship:** stepping in so a family caregiver can rest, and a reliable, watchful presence.
- **Assistance with self-administered medication:** reminding and helping a client take their own medication — never administering it.

## Part 2 — The License You Need

Here is the single most important thing to understand about opening in Ohio: non-medical home care is licensed by the Ohio Department of Health (ODH) under Ohio Revised Code Chapter 3740 and the rules at Ohio Administrative Code Chapter 3701-60. The license you want is the Non-Medical Home Health Services license. Ohio does not divide it into classes or tiers — it is one license covering personal care, homemaking, respite, and assistance with self-administered medication.

The other ODH home-care license is the Skilled Home Health Services license, for skilled nursing and physical, occupational, or speech therapy. You do not need it for a non-medical agency — and if you ever do hold a skilled license, it also covers non-medical services, so you would not need both. Providing skilled care without the skilled license, or providing any home care without the license you need, is a criminal offense in Ohio, so match your license to your services from the start.

### **Who needs the license — and who is exempt**

**If you provide hands-on personal care and homemaking in clients' homes, you need the license.** The license requirement reaches agencies and even self-employed “non-agency” providers who serve more than two individuals, and anyone routinely alone with a client or with access to a client’s property or money. Ohio exempts certain providers — nursing homes, residential care and assisted living facilities, hospices, and providers certified by the Department of Developmental Disabilities, among others. If you are unsure whether an arrangement is exempt, confirm with ODH before you rely on the exemption.

One point of confusion is worth clearing up, because you will see it repeated online: some older guides claim Ohio does not license non-medical home care. That was true before the law changed, but it is wrong now. Since October 2022, ODH licenses non-medical home health services, and the Department’s own application forms and rules confirm it. Do not rely on out-of-date advice that tells you no license is needed — build your plan on the current requirement, and you will not be caught short when a hospital, a payer, or ODH checks.

## Part 3 — Getting Licensed by ODH

You apply for the Non-Medical Home Health Services license through the Ohio Department of Health, Bureau of Regulatory Operations, Licensure Program. ODH suggests allowing about ninety days, so build the timeline into your plan and do not accept clients or provide services before the license is issued.

### What the application involves

- **The application and fee.** Submit ODH's Non-Medical Home Health Services license application with the \$250 non-refundable fee (payable to "Treasurer, State of Ohio").
- **Owner fingerprints.** The primary owner provides a fingerprint impression card (not a criminal record check) to ODH.
- **A criminal-records-check policy.** Provide your written policy for conducting the caregiver background checks the rules require.
- **A description of services and your policies.** Describe the non-medical services you will provide, and submit your policy and procedure manual.
- **A surety bond.** An applicant that was not providing direct care on or before September 30, 2021 posts a \$20,000 surety bond for non-medical services.
- **An Ohio office.** Maintain an Ohio administrative office where your records are kept; an out-of-state agency must establish one before applying.

Be realistic about the timeline. Between assembling a complete application, the roughly ninety-day review, background-check processing, and any follow-up ODH requests, several months from first paperwork to an issued license is normal. Use that time productively: it is exactly when you build your caregiver pipeline, finish your policies, and line up your first referral relationships, so that the day your license issues you are ready to take clients rather than starting from zero. The agencies that open fastest are the ones whose application, policies, and caregiver roster all tell the same complete, consistent story.

#### Build to the rule, and keep the license current

Ohio's non-medical rules are lighter than some states' — they center on your application, your caregiver background checks, and your recordkeeping — but they are real, and operating without the license is a criminal offense. Build your policy manual and your caregiver files to ORC 3740 and OAC 3701-60 from the start, renew the license before it expires, keep your bond and insurance current, and stay ready for an ODH review or complaint investigation at any time.

## Part 4 — Caregiver Background Checks

Ohio's non-medical license is built around caregiver screening, so getting this right is most of your compliance. Get it right and everything else follows; get it wrong and it is both a licensing problem and a safety risk.

- **BCI (and FBI) records checks.** The agency requests a Bureau of Criminal Investigation criminal records check — and an FBI check where required — for every direct-care applicant. Request it no later than five business days after a conditional hire begins, and supervise the person until it clears.
- **Disqualifying offenses.** Anyone with a disqualifying offense is barred from a direct-care position, subject only to the narrow exceptions Ohio law allows.
- **Database reviews.** Review the state and federal databases and abuse and exclusion registries the rules require before hire, and screen against Medicaid exclusion lists monthly if you serve Medicaid clients.
- **The roster and re-checks.** Keep a confidential roster showing when each background check was requested and received and the determination, and re-check each direct-care employee at least every five years.

Because background checks take time and Ohio gives you a five-business-day window, start the check the moment you make a conditional offer, keep the roster current, and never let a caregiver establish a routine, unsupervised relationship with a client before the check clears.

Recruiting is the constant work of this business, so treat your caregiver pipeline as a system, not a scramble. Advertise steadily, respond to applicants fast, and move good candidates through offer, background check, orientation, training, and competency without letting them cool off — the best caregivers have options. Pay competitively for your area, treat people well, and build a culture caregivers want to stay in, because every caregiver you keep is one you do not have to recruit, screen, and train again. A reliable bench of screened, trained, competency-evaluated caregivers is what lets you say yes to a new client or referral on short notice — which is exactly what referral sources remember.

## Part 5 — Orientation, Training & Competency

Ohio expects you to prepare and evaluate every caregiver before they work independently. The license rules focus on screening, but good practice — and the Medicaid waiver programs, if you serve their clients — require real training and competency.

- **Orientation.** Before serving clients independently, each caregiver completes an agency orientation covering your policies, the non-medical scope, client rights, abuse reporting, personal care and homemaking, assisting with self-administered medication, infection control, and safety.
- **Initial training.** Training matched to your services — safe personal-care and transfer techniques, preventing skin breakdown and falls, communication, and population-specific topics such as dementia care.
- **Competency evaluation.** Evaluate each caregiver's competency in the assigned tasks by observation and review before independent service, and again for new tasks or after a concern.
- **Medicaid training.** If you serve Medicaid waiver clients, complete the orientation and training the applicable program requires, and the EVV training the provider enrollment requires.

### Document everything, in every file

Ohio's screening and your training and competency are only as good as your records. Document each caregiver's background check (request and receipt dates and the determination) on your roster, and their orientation, training, and competency in their personnel file. At a review, ODH will expect to see that every direct-care person was screened before client contact and that your roster proves it.

Training is also where you win or lose in Ohio's tight caregiver labor market. The agencies that keep good caregivers make orientation and training feel like real preparation and respect rather than a box to check — a caregiver who starts confident and supported is safer, stays longer, and costs you less in turnover. Build your training so it genuinely prepares people for the homes and clients they will serve, evaluate competency honestly, and keep investing in your caregivers after hire; it is both good practice and your best retention strategy.

## Part 6 — Setting Up to Operate

### Form the business and name an administrator

Register an LLC or corporation with the Ohio Secretary of State, get a federal EIN, and open a business bank account. Name an administrator responsible for running the agency and its compliance with ODH — give the role real authority and time. Ohio has no statewide sales tax on these services, but you still meet all Ohio and federal tax and employment obligations.

### Insurance, bond, policies, and supervision

Carry professional and general liability insurance and workers' compensation coverage, and obtain the surety bond the license requires. Adopt a policy and procedure manual, client forms, and a caregiver handbook built to ORC 3740 and OAC 3701-60. Assess each client, write a service plan, review it with each assigned caregiver before care begins, and supervise service delivery — the service plan and supervision are your quality-control system.

Your administrator is the linchpin. ODH holds the agency accountable through this role, so give it real authority and time — an administrator stretched too thin or existing only on paper is both a compliance risk and the reason quality slips. The administrator owns your compliance calendar (license renewal, background rechecks, training currency, insurance, and the bond), your service-planning and supervision systems, and your quality program that tracks complaints, incidents, and satisfaction and turns them into fixes. Build these systems small and real from the start; it is far easier to grow a disciplined two-caregiver agency into a large one than to impose discipline on a chaotic one later.

## Part 7 — Finding Clients & Referrals

Two engines fill your schedule: families who pay privately, and the aging-and-disability network that steers people to agencies. Build both from the start.

- **Hospital discharge planners and case managers** send people home who need help — be the licensed agency they trust to prevent a readmission.
- **Skilled nursing and rehab facilities** discharge clients who still need in-home support after therapy ends.
- **Hospices** refer both ways: they need aides for personal care, and you refer declining clients to them.
- **Assisted living and senior communities** have residents who need one-on-one help beyond what the community provides.
- **Area Agencies on Aging and the PASSPORT administrative agencies** route Ohio families and Medicaid clients to services.
- **Elder-law attorneys and geriatric care managers** guide families to trustworthy licensed agencies.

Your PolicyReady Referral System for Ohio lists named hospitals, skilled nursing facilities, hospices, senior communities, elder-law attorneys, and care managers across Columbus, Cleveland, Cincinnati, Dayton, and Toledo, with the playbook for turning them into a steady stream of clients.



## Part 8 — Getting Paid

In-home care in Ohio is paid three main ways: privately by the client or family; by Ohio Medicaid through its home- and community-based services waivers; and by other sources such as long-term-care insurance and Veterans benefits. Most new agencies start with private pay — Ohio has a solid private-pay market — and add Medicaid once established.

### Private pay

Set a clear hourly rate, put it in a written service agreement, and bill only for services actually delivered. Private pay needs no Medicaid enrollment and is the fastest way to open once you are licensed. Ohio's lack of a sales tax helps; build in your real labor costs, including overtime and Ohio's minimum wage, from the start.

### Ohio Medicaid waivers

Ohio funds in-home care through several Medicaid home- and community-based services waivers: PASSPORT (older adults, administered by the Department of Aging through regional PASSPORT administrative agencies), the Ohio Home Care Waiver (younger adults with disabilities, administered by the Department of Medicaid), and MyCare Ohio (dual Medicare-Medicaid eligibles, through managed care). To serve waiver clients you enroll as a provider — currently through Public Consulting Group (PCG) — complete EVV training, capture visits through Ohio's EVV system (Sandata), and provide only authorized services. Provider enrollment is separate from your ODH license.

Think about payer mix deliberately. Private pay is faster to start, pays more per hour, and gives you scheduling flexibility, but it depends on families who can afford it. The Medicaid waivers open a much larger client base and steadier referral flow through case managers, but they pay set rates, add authorization and EVV requirements, and depend on enrollment being open. Many successful Ohio agencies start private-pay to get licensed and stable, then add waiver clients as enrollment allows, and later layer on long-term-care insurance and Veterans' benefits. Whatever the mix, price from your real costs — caregiver wages at or above Ohio's minimum, overtime, workers' compensation, insurance, the bond, and supervision — and revisit your rates each year as the minimum wage rises.

#### Two Ohio realities to plan around

**A provider moratorium and EVV.** Ohio has at times placed a statewide moratorium on adding new Medicaid waiver agency providers, so confirm current enrollment availability before you count on Medicaid revenue — your ODH license and private pay do not depend on it. And every Medicaid in-home visit must be captured through Electronic Visit Verification (Sandata); set up EVV and complete the training before you take waiver clients.

## Part 9 — Your First 90 Days

1. **Before you apply:** Form the LLC, get your EIN and bank account, name your administrator, and bind insurance, workers' comp, and the surety bond.
2. **Before you apply:** Adopt your policy manual, client forms, and caregiver handbook to ORC 3740 and OAC 3701-60, and build your screening and training systems.
3. **Apply to ODH:** Submit the application, fee, bond, owner fingerprints, criminal-records-check policy, and service description; allow about ninety days.
4. **During review:** Recruit caregivers, request their BCI checks within five business days of hire, and complete orientation, training, and competency.
5. **After licensure:** Take your first private-pay clients, run real supervisory oversight, begin Medicaid provider enrollment if available, and start referral outreach to two or three hospitals or hospices.

## Part 10 — Common Mistakes to Avoid

- **Assuming Ohio doesn't license non-medical care.** It does — some out-of-date guides say otherwise. You need the Non-Medical Home Health Services license before serving clients.
- **Serving clients before the license issues.** Operating without the license is a criminal offense; plan your timeline around the ~90-day review.
- **Missing the five-business-day background-check window.** Request the BCI check within five business days of a conditional hire and supervise until it clears.
- **Forgetting the surety bond.** A new non-medical applicant posts a \$20,000 bond — line it up before you apply.
- **Treating Medicaid enrollment as automatic.** It is separate from your license and may be under a moratorium — verify availability.
- **Skipping EVV setup.** Every Medicaid visit must be captured through Sandata; set it up before taking waiver clients.
- **Providing skilled care beyond your license.** Skilled nursing and therapy require the separate Skilled license — otherwise refer it out.

## Part 11 — Staying Compliant & Growing

Once you are open, compliance is ongoing: renew your license on time; keep every caregiver's background check, training, and competency current and re-check backgrounds at least every five years; keep your roster and records complete; complete supervisory oversight and keep service plans updated; maintain insurance and the bond; and run a quality program that tracks complaints, incidents, and satisfaction and fixes what they show. Keep a compliance calendar so nothing lapses, and stay ready for an ODH review at all times.

Growth comes from three moves: deepen referral relationships until you are the agency a hospital or hospice calls first; add payers (layer Medicaid waivers onto private pay as enrollment allows, then long-term-care insurance and VA benefits); and expand your service area and team at a pace your administrator and supervision systems can actually support. In a licensed state, your clean review record and your reputation are the whole game.

As you grow, protect what got you there. Do not let recruiting pressure push you into using caregivers you have not fully screened and trained, do not take clients whose needs exceed the non-medical scope or your capacity, and do not let documentation slip as volume rises — the failures that end agencies almost always trace back to compromising on screening, scope, or records to chase growth. Add caregivers and clients at a pace your administrator and supervision systems can actually support, keep your compliance calendar current, and reinvest in training and retention. Sustainable growth in Ohio looks like a widening base of loyal referral sources and a stable, well-supported caregiver team — not a spike in clients your systems cannot safely carry.

## Part 12 — What It Costs to Start

Your startup budget has a few big categories. Two Ohio-specific fixed costs are the ODH application fee of \$250 and, for a new non-medical applicant, a \$20,000 surety bond (the bond is purchased through a surety company; its cost is a fraction of the face amount and depends on credit). Beyond those, plan for the following, and gather real quotes rather than guessing:

- **Insurance:** professional and general liability and workers' compensation — get quotes for your size and services.
- **Business formation:** LLC or corporation registration with the Ohio Secretary of State, plus your accounting setup.
- **Policies and documents:** your policy and procedure manual, client forms, and caregiver handbook built to ORC 3740 and OAC 3701-60.
- **Caregiver screening and training:** BCI and FBI background checks, database reviews, orientation, training, and competency evaluation for every caregiver.
- **Office and systems:** a small Ohio administrative office, scheduling and documentation software (and Sandata EVV if you serve Medicaid), phones, and supplies.
- **Working capital:** enough cash to cover several months of wages, insurance, and overhead before revenue is steady — the cost new owners most often underestimate, given the licensing timeline.

In-home care is not equipment-heavy, which keeps startup lighter than facility-based care, but the working-capital runway to your license and first clients is real — fund it deliberately rather than assuming revenue will arrive quickly.

## Two Worked Examples

### Example 1 — A private-pay start in Columbus

You form an LLC, name an administrator, bind insurance and workers' comp, and obtain your \$20,000 surety bond. You adopt your policy manual and apply to ODH for the Non-Medical Home Health Services license with the \$250 fee, owner fingerprints, and your criminal-records-check policy. While ODH reviews the application, you hire two caregivers, request their BCI checks within five business days, and put them through orientation, training, and a competency evaluation. Your license issues, and your first client comes from a hospital discharge planner who needs safe help for a patient going home after a fall — you assess the home, write a service plan, and start daily personal care plus medication reminders, private pay, billed weekly. Referral targets you would build in Columbus include hospitals, skilled nursing and rehab facilities, hospices, and senior communities across the metro.

### Example 2 — Adding Medicaid in Cleveland or Cincinnati

Your agency has a steady private-pay base and you are ready for Medicaid. You confirm current provider enrollment availability, enroll through Public Consulting Group, complete EVV training, and set up Sandata — all separate from your ODH license. A PASSPORT administrative agency or Ohio Home Care Waiver case manager authorizes in-home care for an eligible client; you assign a screened, trained, competency-evaluated caregiver, deliver only authorized services per the plan, capture each visit through EVV, and supervise the case. As you take waiver clients, you keep the private-pay side growing through the same referral network — hospitals, skilled nursing, hospices, assisted living, and the Area Agencies on Aging — in Cleveland, Cincinnati, and the surrounding communities.

## Who to Call & What to Ask

| For...                         | Who to contact                                                                                                                                                                                                                                                  |
|--------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Forming your business          | Ohio Secretary of State (ohiosos.gov). Ask: what do I file to register my LLC, and what are the ongoing requirements?                                                                                                                                           |
| Licensing your agency          | Ohio Department of Health, Bureau of Regulatory Operations, Licensure Program (odh.ohio.gov, Home Health Agency Licensure). Ask: what does a complete Non-Medical Home Health Services application require, and what are the current fee, bond, and timeframes? |
| Background checks              | Ohio Bureau of Criminal Investigation, through the Attorney General's office (webcheck locations). Ask: how do my caregivers complete BCI and FBI records checks, and what are the disqualifying offenses?                                                      |
| Becoming a Medicaid provider   | Public Consulting Group (PCG) HCBS provider enrollment — 877-908-1746. Ask: is enrollment open, how do I enroll for PASSPORT, the Ohio Home Care Waiver, or MyCare Ohio, and how do I set up EVV?                                                               |
| The aging & disability network | Your Area Agency on Aging and the Aging and Disability Resource Network, and the Eldercare Locator (1-800-677-1116). Ask: how do families and Medicaid clients find and choose in-home care?                                                                    |
| Wage & hour rules              | Ohio Department of Commerce (minimum wage) and the U.S. Department of Labor. Ask: what are the current Ohio minimum wage and the FLSA home care overtime rules?                                                                                                 |
| Reporting abuse                | The county Department of Job and Family Services / Adult Protective Services (ORC 5101.60-5101.71); child abuse 1-855-642-4453; 911 for emergencies.                                                                                                            |
| Filing or fielding a complaint | Ohio Department of Health home health hotline 800-669-3534 and complaint line 800-342-0553. Ask: how are complaints filed and investigated?                                                                                                                     |



## Key Terms

- **Non-Medical Home Health Services license:** the ODH license a non-medical in-home care agency must hold under ORC 3740 and OAC 3701-60.
- **Skilled Home Health Services license:** the separate, higher ODH license for skilled nursing and therapy — not needed for a non-medical agency.
- **ODH:** the Ohio Department of Health, which issues and oversees the license.
- **BCI:** the Ohio Bureau of Criminal Investigation, which conducts caregiver criminal records checks.
- **Disqualifying offense:** an offense that bars a person from a direct-care position under Ohio law.
- **Direct care:** providing services, being routinely alone with a client, or having routine access to a client's property or money.
- **Service plan:** the written plan of non-medical tasks, reviewed with each caregiver before care.
- **PASSPORT / Ohio Home Care Waiver / MyCare Ohio:** Ohio's Medicaid home- and community-based services waivers that fund in-home care.
- **EVV (Sandata):** Electronic Visit Verification, required for Medicaid in-home visits.

## Core Rules & References

- ORC Chapter 3740 — Ohio home health agency licensure statute (sections 3740.01 to 3740.05, 3740.07, 3740.10 to 3740.11, 3740.99)
- OAC Chapter 3701-60 — ODH home health agency licensure rules (3701-60-01 through 3701-60-10), effective October 1, 2022
- OAC 3701-60-04 to -06; ORC 109.572 — caregiver criminal records checks (BCI/FBI), timing, and re-checks
- OAC 3701-60-03, -10 — application, database reviews, and the applicant-and-employee roster
- OAC Chapters 5160-45, 5160-31, 5160-46, 5160-58 — Ohio Medicaid HCBS waivers (general, PASSPORT, Ohio Home Care, MyCare Ohio)
- ORC 5101.60 to 5101.71 — reporting abuse, neglect, or exploitation of an adult; child abuse 1-855-642-4453; the FLSA home care rule and Ohio wage law

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*This guide is a general educational resource for prospective Ohio in-home care agency owners. It is not legal, tax, or licensing advice, and requirements, fees, and contacts change. Verify every step, figure, and phone number with the agency named — the Ohio Secretary of State, the Ohio Department of Health, the Bureau of Criminal Investigation, and the relevant payer — before you rely on it. Provisions cited reflect Ohio law and ODH rules at the time of writing.*